Adult Failed Airway

AR-02 | Airway & Respiratory

Quick flow

01 Declare Failed Airway [EMT / AEMT / Paramedic]

Cannot oxygenate/ventilate • unsuitable anatomy • or three total failed attempts.

02 Call for Resources [EMT / AEMT / Paramedic]

Bring the most experienced airway provider and rescue devices.

03 BVM + Adjuncts [EMT / AEMT / Paramedic]

Optimize position, suction, mask seal, OPA/NPA, PEEP, and two-person technique.

04 Oxygenation Target Met? [EMT / AEMT / Paramedic]

SpO_2 at least 92%, or at least 94% for suspected TBI: continue effective BVM. If oxygenation or ventilation is ineffective, place an authorized rescue airway.

05 Rescue Airway [AEMT / Paramedic]

AEMT/Paramedic: authorized BIAD. Paramedic: changed video-laryngoscopy rescue or surgical airway only when required.

06 Suspected TBI? [EMT / AEMT / Paramedic]

SpO_2 ≥94%; urgently correct <90% or inadequate ventilation. Initial/routine assisted $EtCO_2$ 35-45 mmHg. No routine hyperventilation. Clear active/imminent herniation only: brief $EtCO_2$ 30-35 with capnography, avoid <30, and expedite transport. Open TB-05.

07 Confirm + Transport [EMT / AEMT / Paramedic]

Waveform $EtCO_2$, secure device, post-airway care, and early notification.

Overview

- Recognize failure early, stop repeated laryngoscopy, and return immediately to the most effective oxygenation method.
- BVM with adjuncts is acceptable definitive field management when it maintains oxygenation and ventilation.

Provider-Level Actions

Use the least invasive effective airway intervention and escalate only when oxygenation or ventilation remains inadequate.

EMT

- Stop repeated attempts, optimize two-person BVM with airway adjuncts, suction, oxygen, and PEEP when indicated.
- Continue BVM when it maintains oxygen saturation at least 92% (at least 94% for suspected TBI) and adequate ventilation.

AEMT

- Perform all EMT care plus authorized BIAD placement; do not perform ETT placement or video laryngoscopy under this protocol.
- Confirm a BIAD with waveform capnography and secure it during all movement.

Paramedic

- Perform all prior care and proceed to surgical cricothyrotomy only when less invasive methods cannot oxygenate or ventilate.
- Avoid additional laryngoscopy after three total attempts or when anatomy makes success unlikely.

Indications

- Unable to ventilate and maintain oxygen saturation at least 90% during or after one or more unsuccessful attempts.
- Anatomy inconsistent with continued attempts or three unsuccessful attempts by the most experienced Paramedic.

Treatment steps

- Optimize two-person BVM with positioning, suction, OPA/NPA as appropriate, high-flow oxygen, and PEEP when indicated.
- If BVM maintains SpO_2 at least 92% (at least 94% for suspected TBI) and adequate ventilation, continue it as the definitive field airway and begin transport.
- Place an authorized BIAD when BVM is ineffective or prolonged ventilation is required; confirm and secure it before movement.
- Proceed to surgical cricothyrotomy only for a Paramedic-managed cannot-oxygenate/cannot-ventilate patient after less-invasive rescue measures fail or cannot be used.
- Ventilate approximately 10-12 breaths/min with visible chest rise; use $EtCO_2$ and clinical response to avoid hyperventilation.
- For suspected traumatic brain injury, continuously monitor oxygenation and ventilation. Aim for SpO_2 at least 94%; urgently correct SpO_2 below 90% or inadequate ventilation. The condition-specific TBI target takes priority over a lower general airway or post-ROSC oxygenation target.
- For suspected traumatic brain injury requiring assisted ventilation, use an initial/routine $EtCO_2$ target of 35-45 mmHg. Avoid routine hyperventilation. Only for clear active or imminent cerebral herniation, use brief controlled ventilation targeting $EtCO_2$ 30-35 mmHg with continuous waveform capnography while expediting definitive care; avoid overshooting below 30 mmHg.
- Prevent hypotension in suspected traumatic brain injury; do not use permissive hypotension. For adults, target SBP at least 110 mmHg, including age 70, with frequent reassessment and after treatment. Pediatric TBI blood-pressure targets require age-specific direction from Medical Control until the separately approved pediatric TBI target table is published; do not substitute adult targets or generic pediatric shock thresholds.

Warnings / precautions

- No more than three total intubation attempts; every repeated attempt must change the provider, approach, positioning, or equipment.
- Do not abandon effective BVM merely to obtain an advanced airway.
- Do not delay transport for repeated advanced-airway attempts once oxygenation and ventilation are effective.

Clinical pearls

- The clinical goal is oxygenation and ventilation, not endotracheal intubation.
- Use DOPE for sudden deterioration after airway placement.

Related pathways / contacts

TB-05 Head Trauma - Open the shared TBI oxygenation, ventilation, herniation-rescue, and perfusion targets.

Source and references

Claiborne County EMS approved protocol manual. | Claiborne County EMS formulary, linked protocol pathways, and agency release guidance.

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